

UNIVERSITY MEDICAL CENTRE — ONCOLOGY OUTPATIENT CLINIC

CLINICAL RESEARCH STUDY VISIT NOTE

Study: ZENITH-001 | Visit: Cycle 3 Day 1 | Visit Date: 22 October 2024

PATIENT IDENTIFICATION

Patient Full Name: Margaret Anne Fitzgerald

Date of Birth: 14 March 1967 | Age: 57 years | Sex: Female

Home Address: 17 Maple Grove Drive, Burlington, Vermont, 05401, United States

Social Security Number: 456-78-9012 | **Medicare ID:** 1EG4-TE5-MK72

Primary Telephone: (802) 555-0193 | Emergency Contact: Thomas Fitzgerald (spouse), (802) 555-0241

Primary Care Physician: Dr. Sandra Okonkwo, Burlington Internal Medicine Associates, 211 College Street, Burlington, VT

Subject ID: SUB-017 | Randomisation Number: ZNT-2024-017

REASON FOR VISIT

Patient presents for Cycle 3 Day 1 study visit as part of the ZENITH-001 Phase II trial. She is accompanied by her husband. She reports feeling 'mostly okay but having some stomach issues' over the past two weeks.

INTERVAL HISTORY

Since her last visit on 24 September 2024 (Cycle 2 Day 1), Ms. Fitzgerald reports a difficult cycle. She states she has been attempting to take her study medication every morning but has missed approximately 5 doses over the past 28 days due to nausea. She completed the patient diary, which has been collected for data entry.

Current Medications (as reported by patient this visit):

- Veridaxinib (VDX-114) 200 mg oral daily — study drug (per protocol)
- Ondansetron 8 mg oral as needed for nausea — per protocol antiemetic
- Methotrexate 15 mg oral once weekly — patient states she has been taking this for the past 6 months for her rheumatoid arthritis, prescribed by Dr. Okonkwo. Patient reports she did not disclose this at screening as she 'did not think it counted as a cancer drug'.
- Folic acid 1 mg daily — taken with methotrexate as prescribed by PCP
- Lisinopril 10 mg daily — for hypertension

REVIEW OF SYSTEMS

General: Patient reports fatigue, rated 5/10 in severity. Weight loss of approximately 3 kg since last visit is noted (unintentional per patient report). No fever or chills.

Gastrointestinal: Patient reports severe nausea beginning approximately 3 days after Cycle 2 commencement and persisting through the current visit date. She describes vomiting 4 to 5 episodes daily, occurring both with and without meals, with the worst episodes in the morning. She has been unable to maintain consistent oral fluid intake and describes eating only soup and crackers for the past 10 days. She has used ondansetron daily without adequate relief.

Respiratory: Patient denies any shortness of breath, cough, or chest pain at rest or on exertion.

Cardiovascular: No palpitations, no chest tightness, no oedema of lower extremities reported.

Musculoskeletal: Mild bilateral wrist and finger joint aching, which patient attributes to her underlying rheumatoid arthritis.

PHYSICAL EXAMINATION

Vital Signs: Blood Pressure 108/72 mmHg (notably lower than baseline 128/84). Heart Rate 94 bpm. Temperature 37.4 degrees Celsius. Respiratory Rate 16 breaths/min. Oxygen Saturation 97% on room air. Weight 61.2 kg (previously 64.3 kg at Cycle 2 Day 1, a loss of 3.1 kg over 28 days).

General: Patient appears mildly distressed, pale, and dehydrated. Mucous membranes dry. Skin turgor mildly reduced.

Chest and Lungs: Breath sounds clear bilaterally. Patient demonstrates mildly laboured breathing on ambulating from waiting room to examination room. She reports mild dyspnea on exertion over the past week, stating 'I get winded going up one flight of stairs, which is new for me.' She had not mentioned this when directly asked about respiratory symptoms earlier in the interview.

Abdomen: Soft, mildly tender in epigastric region. No hepatosplenomegaly appreciated. Bowel sounds present and active.

Extremities: No peripheral oedema. Mild synovial thickening of bilateral PIP and MCP joints.

LABORATORY RESULTS — CYCLE 3 DAY 1

Test	Result	Reference Range	Flag
Haemoglobin	9.8 g/dL	12.0–16.0 g/dL	LOW — Grade 2
WBC	3.1 x10 ⁹ /L	4.0–11.0 x10 ⁹ /L	LOW
Platelets	142 x10 ⁹ /L	150–400 x10 ⁹ /L	LOW
ALT (SGPT)	187 U/L	7–40 U/L	HIGH — Grade 3 (4.7x ULN)
AST (SGOT)	203 U/L	10–40 U/L	HIGH — Grade 3 (5.1x ULN)
Total Bilirubin	1.9 mg/dL	0.2–1.2 mg/dL	HIGH — Grade 1

Serum Creatinine	1.1 mg/dL	0.5–1.1 mg/dL	Normal
Albumin	2.9 g/dL	3.5–5.0 g/dL	LOW
Sodium	133 mEq/L	135–145 mEq/L	LOW

ASSESSMENT AND PLAN

1. Grade 3 Nausea/Vomiting: Consistent with treatment-related gastrointestinal toxicity. Ondansetron has provided inadequate relief. Will prescribe IV hydration (1 litre normal saline) today in the infusion suite and consider adding prochlorperazine 10 mg oral TID as second-line antiemetic.
2. Grade 3 Hepatic Transaminase Elevation: ALT and AST consistent with Grade 3 hepatotoxicity per CTCAE v5.0. Likely multifactorial — Veridaxinib hepatotoxicity versus methotrexate hepatotoxicity versus concurrent ethanol exposure not yet investigated. Study drug to be held pending Medical Monitor review.
3. Prohibited Concomitant Medication: Methotrexate use is a protocol violation under Section 4.2(b) as a prohibited immunosuppressant. Protocol Deviation Report to be completed. Medical Monitor notified verbally today.

No adverse events of clinical significance were observed or reported during this study visit.

Electronically signed: Dr. Jonathan P. Chen, MD, PhD | Investigator | University Medical Centre Oncology

Date/Time of Note Completion: 22 October 2024 at 16:47 EST